

FLOOR AMENDMENT

HOUSE OF REPRESENTATIVES

State of Oklahoma

SPEAKER:

CHAIR:

I move to amend HB3644 _____
Of the printed Bill
Page _____ Section _____ Lines _____
Of the Engrossed Bill

By deleting the content of the entire measure, and by inserting in lieu thereof the following language:

AMEND TITLE TO CONFORM TO AMENDMENTS

Adopted: _____

Amendment submitted by: Preston Stinson

Reading Clerk

STATE OF OKLAHOMA

2nd Session of the 60th Legislature (2026)

FLOOR SUBSTITUTE

FOR

HOUSE BILL NO. 3644

By: Stinson

FLOOR SUBSTITUTE

An Act relating to venous thromboembolisms screening and treatment; creating the Blake Burgess Act; requiring certain hospitals to develop and implement policies and procedures regarding venous thromboembolisms; mandating training for the rendering of appropriate medical attention for persons at risk of forming venous thromboembolisms; requiring the State Department of Health to contract with a private entity to establish a statewide venous thromboembolism registry; providing requirements; requiring hospitals to report certain information regularly to the statewide venous thromboembolism registry; requiring the private entity to provide regular reports to the Department on such data; requiring the Department to provide to the Governor and the Legislature a specified report; providing requirements for report; providing applicability; amending 63 O.S. 2021, Section 1-890.2, which relates to definitions in the Continuum of Care and Assisted Living Act; adding definitions; amending 63 O.S. 2021, Section 1-890.3, as amended by Section 1, Chapter 357, O.S.L. 2025 (63 O.S. Supp. 2025, Section 1-890.3), which relates to promulgation of necessary rules, nursing care component, and adult daycare component; requiring assisted living facilities to provide a consumer information pamphlet containing specified information to residents; providing for noncodification; providing for codification; providing an effective date; and declaring an emergency.

BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

SECTION 1. NEW LAW A new section of law not to be codified in the Oklahoma Statutes reads as follows:

This act shall be known and may be cited as the "Blake Burgess Act".

SECTION 2. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 1-630 of Title 63, unless there is created a duplication in numbering, reads as follows:

Each hospital with an emergency department and each ambulatory surgical center shall:

1. Develop and implement policies and procedures for the rendering of appropriate medical attention for persons at risk of forming venous thromboembolisms which reflect evidence-based best practices relating to, at a minimum:

a. assessing patients for risk of venous thromboembolism using a nationally recognized risk assessment tool, and

b. treatment options for a patient diagnosed with venous thromboembolism; and

2. Train all nonphysician personnel at least annually on the policies and procedures developed under this section. For purposes of this section, the term "nonphysician personnel" means all personnel of the licensed facility working in clinical areas and

1 providing patient care, except those persons licensed as health care
2 practitioners. In no case shall this be construed to require
3 personnel to practice outside the scope of his or her certification,
4 license, or qualification.

5 SECTION 3. NEW LAW A new section of law to be codified
6 in the Oklahoma Statutes as Section 1-631 of Title 63, unless there
7 is created a duplication in numbering, reads as follows:

8 A. The State Department of Health shall contract with the
9 state-designated health information exchange to establish and
10 maintain a statewide venous thromboembolism registry to ensure that
11 the performance measures required to be submitted under subsection B
12 of this section are maintained and available for use to improve or
13 modify the venous thromboembolism care system, ensure compliance
14 with nationally recognized guidelines, and monitor venous
15 thromboembolism patient outcomes.

16 B. Beginning July 1, 2027, each hospital with an emergency
17 department shall annually report to the statewide venous
18 thromboembolism registry information containing nationally
19 recognized venous thromboembolism measures and data on the incidence
20 and prevalence of venous thromboembolisms. Such data shall include
21 the following information:

22 1. The number of venous thromboembolisms identified and
23 diagnosed;

24 2. The age of the patient;

1 3. The ZIP code of the patient;

2 4. The sex of the patient;

3 5. Whether the patient is a resident of a licensed nursing or
4 assisted living facility;

5 6. Whether the venous thromboembolism was fatal;

6 7. How the diagnosis was made, such as by using imaging
7 modalities; and

8 8. The treatment that was recommended for the venous
9 thromboembolism.

10 C. The Department shall require the state-designated health
11 information exchange to collect data from each hospital with an
12 emergency department on the performance measures required under
13 subsection B of this section. The state-designated health
14 information exchange shall provide to the Department regular reports
15 on the data collected.

16 D. By June 1, 2027, the Department shall submit to the
17 Governor, the President Pro Tempore of the Oklahoma State Senate,
18 and the Speaker of the Oklahoma House of Representatives a detailed
19 report on incidents of venous thromboembolism using inpatient and
20 outpatient data for services provided between July 1, 2026, and June
21 30, 2027. The report shall provide analyses of all of the
22 following:

23 1. Age category, initial primary diagnosis and procedure, and
24 secondary diagnoses, readmission rates for inpatients, admission

1 rates for venous thromboembolism for which the patient had an
2 ambulatory surgery procedure, and emergency department visits for
3 venous thromboembolism linked to any previous admission;

4 2. Whether the venous thromboembolism was present upon
5 admission;

6 3. The incidence of venous thromboembolism procedures reported
7 on the agency's website; and

8 4. The principal payor, the sex of the patient, and the
9 patient's discharge status.

10 E. The state-designated health information exchange operating
11 the registry shall only use or publish information from the registry
12 for the purposes of advancing medical research or medical education
13 in the interest of reducing morbidity or mortality.

14 SECTION 4. AMENDATORY 63 O.S. 2021, Section 1-890.2, is
15 amended to read as follows:

16 Section 1-890.2. As used in the Continuum of Care and Assisted
17 Living Act:

18 1. "Assisted living center" means any home or establishment
19 offering, coordinating or providing services to two or more persons
20 who:

- 21 a. are domiciled therein,
- 22 b. are unrelated to the operator,
- 23 c. by choice or functional impairments, need assistance
24 with personal care or nursing supervision,

- d. may need intermittent or unscheduled nursing care,
- e. may need medication assistance, and
- f. may need assistance with transfer and/or ambulation;

2. "Board" means the State Board of Health;

3. "Commissioner" means the Commissioner of Health;

4. "Continuum of care facility" means a home, establishment or institution providing nursing facility services as defined in Section 1-1902 of this title and one or both of the following:

a. assisted living center services as defined in the Continuum of Care and Assisted Living Act, and

b. adult day care center services as defined in Section 1-872 of this title; ~~and~~

5. "Department" means the State Department of Health;

6. "Pulmonary embolism (PE)" means a condition in which part of the clot breaks off and travels to the lungs, possibly causing death; and

7. "Venous thromboembolism (VTE)" means deep vein thrombosis (DVT), which is a blood clot located in a deep vein, usually in the leg or arm. The term can be used to refer to DVT, pulmonary embolism, or both.

SECTION 5. AMENDATORY 63 O.S. 2021, Section 1-890.3, as amended by Section 1, Chapter 357, O.S.L. 2025 (63 O.S. Supp. 2025, Section 1-890.3), is amended to read as follows:

1 Section 1-890.3. A. The State Commissioner of Health shall
2 promulgate rules necessary to implement the provisions of the
3 Continuum of Care and Assisted Living Act. Such rules shall
4 include, but shall not be limited to:

5 1. A uniform comprehensive resident screening instrument to
6 measure the needs and capabilities of residents in all settings and
7 to determine appropriate placements of residents. Such assessment
8 shall include recognizing the signs and symptoms of venous
9 thromboembolism (VTE) and techniques for providing an emergency
10 response;

11 2. Physical plant requirements meeting construction and life
12 safety codes, with provisions accommodating resident privacy and
13 independence in assisted living centers and in assisted living
14 components of continuum of care facilities based on the variable
15 capabilities of residents;

16 3. Staffing levels responsive to the variable needs of
17 residents, with provisions for sharing of staff between components
18 in a continuum of care facility;

19 4. Minimum standards for resident care including, but not
20 limited to, standards pertaining to medical care and administration
21 of medications. Standards pertaining to medication administration
22 shall, at a minimum, require the assisted living center or continuum
23 of care facility to:

- a. provide or arrange qualified staff to administer medications based on the needs of residents,
- b. follow medication administration orders from a qualified health care provider,
- c. ensure that medications are reviewed monthly by a Registered Nurse or pharmacist and quarterly by a consultant pharmacist,
- d. maintain medication administration records and document all medication administration in such records, and
- e. have medication storage and disposal policies;

5. Standards for measuring quality outcomes for residents;

6. Provisions for individualized services chosen by and designed for each resident;

7. Provisions to prohibit facility staff from disclosing a resident's financial information to third parties without written consent of the resident or the designated representative of the resident;

8. Procedures for inspections and investigations of licensed entities to ensure compliance with the Continuum of Care and Assisted Living Act and rules promulgated by the Commissioner;

9. Enumeration of resident rights and responsibilities to be observed by each facility and its staff. Such resident rights shall include the freedom of choice regarding any personal attending

1 physicians and all other providers of medical services and supplies,
2 providing that the minimum standards are met by the provider
3 pursuant to the Continuum of Care and Assisted Living Act, without a
4 financial penalty or fee charged by the assisted living center;

5 10. Provisions for a surety bond or deposit from each applicant
6 in an amount sufficient to guarantee that obligations to residents
7 will be performed, with provisions for reduction or waiver of the
8 surety bond or deposit when the assets of the applicant or its
9 contracts with other persons are sufficient to reasonably ensure the
10 performance of its obligations;

11 11. Assisted living facilities shall provide a consumer
12 information pamphlet to residents upon admission. The pamphlet
13 shall contain information about venous thromboembolism (VTE), risk
14 factors, and how residents can recognize the signs and symptoms of
15 VTE. This shall be provided by the National Blood Clot Association
16 at no cost to the facility;

17 12. Provisions for the development of a consumer guide or
18 similar resource to be posted on the Internet website of the State
19 Department of Health to assist individuals and families in
20 understanding the services provided by assisted living centers and
21 to compare and select a facility;

22 ~~12.~~ 13. Provisions for posting results of routine inspections
23 and any complaint investigations of each assisted living center on
24 the Internet website of the Department. Such information shall be

1 regularly updated to include the facility's plan of correction and
2 to indicate when a violation of a licensing regulation was corrected
3 by the facility; and

4 ~~13.~~ 14. Provisions requiring execution of a plan of care and a
5 resident service contract with the resident or resident's
6 representative.

7 B. The nursing care service of a continuum of care facility
8 shall be subject to the requirements, procedures and remedies set
9 out in the Nursing Home Care Act, including provisions relating to
10 resident rights.

11 C. The adult day care component of a continuum of care facility
12 shall be subject to requirements and procedures specified under the
13 Adult Day Care Act.

14 SECTION 6. This act shall become effective July 1, 2026.

15 SECTION 7. It being immediately necessary for the preservation
16 of the public peace, health or safety, an emergency is hereby
17 declared to exist, by reason whereof this act shall take effect and
18 be in full force from and after its passage and approval.

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